



Name: David Reeves | DOB: 9/25/1978 | MRN: 19867099 | PCP: Jayantilal D Bhimani, MD | Legal Name: David Reeves

Distance Health - Feb 11, 2026

with Michael Saribalas, DO at Neurology

Notes from Care Team

Progress Notes by Michael Saribalas, DO at 2/11/2026 9:00 AM



Cleveland Clinic Sleep Disorders Center
Follow up/ Established patient visit

Date of last visit : 5/22/2025

I have communicated my name and active licensure. The patient's identity and physical location were verified at the time of this visit. Either the patient or their legal representative has been informed of the risks and benefits of -- and alternatives to -- treatment through a remote evaluation and consents to proceed with the evaluation remotely.

NOTE:

This medical visit is being completed virtually via secure Zoom Audio and Video Communications Technology Company provided by the Cleveland Clinic Foundation (CCF). Consent related to this virtual visit being carried out was provided via Epic MyChart as well as verbally. My findings and recommendations will be transmitted electronically via shared medical records to the consulting provider.

I have communicated my name and active licensure. The patient's identity and physical location were noted at the time of this visit. Either the patient or their legal representative has been informed of the risks and benefits of -- and alternatives to -- treatment through a remote evaluation and consents to proceed with the evaluation remotely.

This patient presents today to the Cleveland Clinic Neurological Institute Sleep Disorders Center Main Campus or virtually for a Sleep Medicine appointment. The medical records available in our Epic electronic medical record system have been reviewed and pertinent information related to this specific visit are included in this manually typed note. This information will be available to the referring health care provider as well as the patient in the Epic MyChart EMR system.

The patient returns today virtually for a scheduled medical visit in the Neurological Institute Sleep Disorders Center. All pertinent information in the medical record for the interval between the last visit in the Sleep Disorders Center and today have been reviewed. The last clinical visit if applicable in the Sleep Disorders Center is attached below.

Distance Health

11/7/2025

NEUROLOGY

Moore,
Kathleen E,

Hypersomnia Follow Up
due to medical Reason for

APRN.CNP condition +4 Visit
Sleep Medicine more
Dx

Progress Notes

Moore, Kathleen E, APRN.CNP (Nurse Practitioner) • Sleep Medicine

[Expand All](#) [Collapse All](#)



Cleveland Clinic Sleep Disorders Center

Follow up/ Established patient visit

Recording using ambient AI software for draft documentation of the visit was discussed with the patient/ authorized representative; all questions welcomed and answered. Patient/authorized representative agreed to proceed

I have communicated my name and active licensure. The patient's identity and physical location were verified at the time of this visit. Either the patient or their legal representative has been informed of the risks and benefits of -- and alternatives to -- treatment through a remote evaluation and consents to proceed with the evaluation remotely.

Assessment/Plan from last visit:

Date of last visit : 7/24/25

IMPRESSION/PLAN:

David Reeves is a 46 yo M with PMH of schizoaffective disorder and hypersomnia disorder possibly narcolepsy with cataplexy, RLS DM2, HTN, HLD resident of an assisted living house. Here for FU.

He reported improvement in nighttime sleep disruption with the higher dose of Lumryz (6 g nightly) that was started 1 month ago. Reported improvement in the loose dreams he described last time however has persistence of daytime sleepiness. He denied cataplexy sleep attacks. He continues to take Adderall as well 15mg ER in the AM and 10mg IR BID every morning plus noon. We discussed increasing Lumryz as a trial however he is still hesitant, he would like to assess if symptoms will improve with caffeine during the day first

Narcolepsy

Plan

- Continue Lumryz 6g QHS
- Adderall XR 15mg q am
- Adderall IR 10mg po bid (one in the AM and then one around noon)
- At upcoming appointment, will consider trial of increasing lumryz up to 7.5 mg rather than increasing Adderall to assess improvement
- Try to get at least 7-9 hours of sleep in a 24 hour period. Healthy diet and exercise can also promote better sleep.

- Monitor for respiratory depression, seizures, decreased consciousness and coma

RLS

He reports worsening of RLS symptoms, has been taking Requip over past 1-1/2 years. Likely experiencing augmentation. Reports symptoms of 8 to 9 PM are the worst, ferritin last checked was 2014 was 263.

Plan

- Start Gabapentin at 100 mg 2 hours before onset of RLS symptoms, Increase by 100 mg every 5 days until reaching relief or side effect or maximum of 600 mg
- Monitor for dizziness, lightheadedness, unsteadiness, sleepiness those symptoms might improve after first week of using
- Check CBC +Iron Studies today
- In the interim continue taking Requip as prescribed
- Plan for tapering Requip when has stable symptoms for concern of Augmentation
- Will be trying to prescribe NIDRA device
- Will prescribe NIDRA device
- Follow up in 2 months

Abdallah Almegdadi, MD

Sleep Medicine Fellow, Sleep Disorders Center

Discussed with Dr.Sikawat

Attending Note

I evaluated the patient and personally participated in the key components of the history and physical exam. I have modified the note above to reflect my clinical decision making. I agree with the resident/fellow's findings and plan as documented and have discussed the case and management of the patient's care with the resident/fellow.

Sikawat Thanaviratananich, MD, MSc

Sleep Medicine

July 25, 2025

3:56 PM

CURRENT VISIT: 11/07/2025

INTERVAL HISTORY:

47 year old male with history of schizoaffective disorder and hypersomnia disorder, possibly narcolepsy with cataplexy, diabetes, HTN. HLD presents for follow up. For hypersomnia, he is managed on Lumryz 6g qHS, Adderall ER 15 mg qAM, and Adderal IR 10 mg BID.

At his last office visit 7/24/2025, the above medications were continued. Due to worsening RLS symptoms, gabapentin 100 mg 2 hours before symptom onset added with recommendation to consider increase gradually up to 600 mg. NIDRA device was also prescribed. Iron studies were performed. Ferritin level was 47, hgb 12.7. Iron supplementation was started in mid August.

PSG 3/20/23 showed UARS, total AHI 4.4, though supine

AHI 98.2. RDI 6.0.

MSLT showed severe hypersomnolence MSL 1.3 minutes,
0 SOREMP

On pre testing actigraphy, inadequate sleep duration prior
to MSLT (average was 5.5 hrs), but adequate sleep if naps
are included (7.9).

HYPERSOMNIA :

Hypersomnia disorder, possibly narcolepsy with cataplexy

Patient reports his hypersomnia has been stable on current
regimen

Naps: No

Cataplexy: No

Hypnagogic hallucinations: No

Dream enactment behaviors: No

Sleep related injuries:

Drowsy driving: NA

SLEEP HYGIENE QUESTIONS:

Bedtime : 9:30-10:30p Wake up Time : 7:00-7:30 AM
with alarm

Time it takes to fall sleep : not long after taking Lumryz

Number of times patient wakes up per night : usually none

Estimated total sleep time (in a 24 hour period of time) :

8-10 hours

Naps : not usually

Current medications:

OARRS checked: Yes

Current medications:

-Medication: Lumryz 6g at bedtime

-Timing: 9:30-10:30p

-Side effects: sometimes groggy in the morning. Denies
other ADRs. Mood stable.

Denies etoh use

Does not drive

-Benefit: improved sleep quality

-Last filled: 10/6/25, 30 days

-Medication: Adderall XR 15 mg daily

-Timing: morning

-Side effects: denies

-Benefit: decreased daytime sleepiness, improved daytime
function

-Last filled: 11/2/25, #30/30 days (written by provider at his
nursing home)

-Medication: Adderall IR 10 mg BID

-Timing: morning, along with XR and at lunch

-Side effects: denies

-Benefit: decreased daytime sleepiness, improved daytime
function

-Last filled: 11/2/25, #60/30 days (written by provider at his
nursing home)

RLS

Reports significant improvement in his RLS symptoms since starting daily iron supplementation with vitamin C. Prior to iron therapy, he experienced RLS symptoms almost nightly and during the day, which prevented sleep and required him to get up and walk around, sometimes for hours. He describes the symptoms as occurring when lying down or sitting, relieved only by standing and walking. Since starting iron, his symptoms have become much more bearable and less frequent, though they remain unpredictable and still occasionally disrupt sleep. He denies constipation or stomach issues from the iron.

He started gabapentin prior to the iron, and the gabapentin did not help at all though he remains on it at this time. He requests discontinuation due to lack of efficacy.

Current medications:

- Medication: ropinirole 2 mg
- Timing: at bedtime
- Side effects: augmentation, needed increase in dosage in past to maintain effectiveness
- Benefit: improvement in RLS symptoms

- Medication: gabapentin 400 mg
- Timing: at bedtime
- Side effects: none
- Benefit: no benefit noted with use
- Last filled: 11/6/25 - being prescribed by nursing home provider

- Medication: iron 65 mg + Vit C
- Timing: in the morning
- Side effects: denies
- Benefit: improvement in RLS symptoms

Ferritin

Date	Value	Ref Range	Status
05/06/2014	263.6	18.0 - 300.0 ng/mL	Final

Transferrin Saturation

Date	Value	Ref Range	Status
05/06/2014	16	11 - 46 %	Final

Hemoglobin

Date	Value	Ref Range	Status
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11/11/202	12.4 (L)	13.0 -	Final
3		17.0 g/dL	
09/26/201	14.5	13.0 -	Final
6		17.0 g/dL	

PATIENT-ENTERED QUESTIONNAIRE SLEEP SCORES:**11/7/2025****Sleep Questions**

Reason for visit:	Narcolepsy Restless Legs Syndrome
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Multiple values from one day are
sorted in reverse-chronological order

7/26/2024 7/24/2025 11/7/2025**Epworth Sleepiness Scale**

Score	Incomplete	4 (No clinically significant daytime sleepiness)	Incomplete
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7/26/2024 7/24/2025 11/7/2025**PROMIS CAT Sleep Disturbance**

PROMIS Sleep Disturbance T-Score	63 (moderate) !	47 (within normal limits)	Incomplete
PROMIS Sleep Disturbance Percentile	10	62	

4/5/2024 7/26/2024 7/24/2025**Restless Leg Syndrome**

Score	Incomplete	Incomplete	30 (Severe symptoms) !
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4/5/2024 7/26/2024 7/24/2025**PHQ-9**

Score	8 ↻	7	0
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↻ Data saved with a previous flowsheet

row definition

2/17/2017 2/6/2019 7/24/2025

PROMIS Global Health - (T-Scores - the mean of general population = 50. Five points is a clinically meaningful difference.)

Physical T-Score	39.8	50.8	50.8
Mental T-Score	43.5	48.3	50.8

Allergies

ALLERGIES

Allergen	Reactions
• Penicillins	Hives
<i>Had hives and wheezing</i>	

ROS

Constitutional: (-) decreased appetite
Ears/Nose/Mouth/Throat: (-) snoring
Cardiovascular: (-) palpitations, (-) chest pain
Respiratory: (-) shortness of breath
Gastrointestinal: (-) constipation, (-) abdominal discomfort
Neurological: (+) restless legs, (+) somnolence, (-) headaches, (-) dizziness, (-) sleepwalking
Psychiatric: (-) depressed mood, (-) anxiety

CURRENT MEDICATIONS:

CURRENT MEDICATIONS

• sodium oxybate ER (LUMRYZ) 6 gram granules in packet	Take 1 packet by mouth daily at bedtime.
• hydrALAZINE (APRESOLINE) 25 mg tablet	Take 2 tablets by mouth once daily as needed (in the evening if SBP>150).
• gabapentin (NEURONTIN) 100 mg capsule	Start at 100 mg 2 hours before onset of RLS symptoms, Increase by 100 mg every 5 days until reaching relief or side effect or maximum of 600 mg
• ropinirole ER (REQUIP XL) 2 mg tablet	Take 2 mg by mouth daily at bedtime.
• dextroamphetamine/amphetamine (ADDERALL ORAL)	Take 10 mg by mouth two times a day.
• dextroamphetamine/amphetamine (ADDERALL XR ORAL)	Take 15 mg by mouth once daily.
• aspirin, enteric coated (ASPIRIN, ENTERIC	Take 81 mg by mouth once daily.

COATED) 81 mg EC tablet	
•cyclobenzaprine HCl (CYCLOBENZAPRIN E ORAL)	Take 10 mg by mouth every 8 hours. Every 8 hours as needed
•ergocalciferol, vitamin D2, (DRISDOL ORAL)	Take 1.25 mg by mouth one time a week.
•GUAIFENESIN ORAL	Take 10 mL by mouth as needed.
•loperamide HCl (IMODIUM A-D ORAL)	Take 2 mg by mouth as needed. Do not exceed 8mg in 24 hours.
•loperamide HCl (IMODIUM ORAL)	Take 4 mg by mouth as needed. Do not exceed 8mg in 24 hours.
•INSULIN ASPART SUBCUTANEOUS	Inject 18 Units subcutaneously three times a day before meals. Before meals
•needles, insulin disposable (NOVOFINE 30 MISC)	8 mm as needed. For DM
•pantoprazole sodium (PROTONIX ORAL)	Take 40 mg by mouth once daily.
•ropinirole HCl (ROPINIROLE ORAL)	Take 2 mg by mouth daily at bedtime.
•ondansetron HCl (ZOFTRAN ORAL)	Take 4 mg by mouth every 6 hours as needed.
•insulin aspart, niacinamide, (FIASP FLEXTOUCH U-100 INSULIN) 100 unit/mL (3 mL) pen	Inject 18 units with meals and sliding scale 3
•BASAGLAR KWIKPEN U-100 INSULIN 100 unit/mL (3 mL)	Give 2x daily 72 units twice daily
•tirzepatide (MOUNJARO) 5 mg/0.5 mL pen injector	Inject 5 mg subcutaneously one time a week.
•amphetamine-dextroamphetamine XR (ADDERALL XR) 15 mg capsule	Take 1 capsule by mouth once daily for 90 days.
•dextroamphetamine-amphetamine (ADDERALL) 10 mg tablet	Take 1 tablet by mouth two times a day for 30 days.
•flash glucose sensor (FREESTYLE LIBRE 2 SENSOR) kit	For blood sugar checks 4 times daily. Takes 4 injections of insulin
•atorvastatin (LIPITOR) 80 mg tablet	Take 80 mg by mouth once daily.

•ezetimibe (ZETIA) 10 mg tablet	
•metFORMIN (GLUCOPHAGE) 1,000 mg tablet	Take 1,000 mg by mouth two times a day with meals.
•fenofibrate nanocrystallized (TRICOR) 145 mg tablet	
•omega-3 acid ethyl esters (LOVAZA) 1 gram capsule	
•paliperidone ER (INVEGA) 9 mg 24 hr tablet	
•ABILIFY MAINTENA 400 mg injection	Patient states he is on a lower dose of this medication , taking in pill form orally. Dose being slowly decreased eventually will be off this medication.
•divalproex DR (DEPAKOTE) 250 mg EC tablet	Take 1 tablet by mouth three times daily.
•acetaminophen (TYLENOL) 325 mg tablet	Take 2 tablets by mouth every 6 hours as needed.

PHYSICAL EXAMINATION:

Vital Signs: Deferred due to virtual visit via Zoom.

General appearance: No acute distress

Mental status: awake and alert

Constitutional: Well groomed

Neuro: Normal thought process, clear speech.

Assessment**/Plan**

Long-term current use of stimulant

Restless legs syndrome

Hypersomnia due to medical condition (primary encounter diagnosis)

Disorder of iron metabolism

High risk medication use

Long-term current use of stimulant (Z79.899)

Hypersomnia due to medical condition (G47.14)

- Stable on LUMRYZ 6g at bedtime; benefit noted, patient reports improved sleep quality with minimal nocturnal awakenings. No reported side effects besides occasional grogginess in the morning that is coneracted by Adderall in AM.

- Continue Adderall XR 15 mg in the morning with Adderall IR 10 mg in the morning and at lunchtime; benefits noted, no adverse effects reported. Stimulants are currently being prescribed by providers that see patient at his nursing

home.

- Discussed potential for LUMRYZ to depress respiratory drive; advised monitoring for SDB, may consider repeat sleep study in near future

Restless legs syndrome (G25.81)

- Symptoms significantly improved with daily oral iron supplementation and vitamin C; minimal to no benefit from gabapentin 400 mg qHS. Patient wishes to discontinue the gabapentin due to avoid any additive side effects. Recommend tapering by 100 mg per week until discontinued.
- Recheck iron studies recommended after 3-6 months of iron replacement. Will order iron studies to be performed sometime in mid December to January time frame; instructed to hold iron supplements for 2 days prior to labs.
- Ropinirole at maximum dose with history of diminishing efficacy; discussed risk of augmentation and other potential side effects including impulse control behaviors. Patient is willing to try to taper off ropinirole. decided to wean him off the gabapentin and ensure optimization of iron stores before attempting wean of ropinirole so as to not change too much at once and since currently, symptoms are well controlled. Would consider at next appointment decrease in dosage.
- Discussed possible iron infusion referral if ferritin <100 ng/mL despite oral iron.
- Discussed potential use of the Nidra device for refractory symptoms.
- Nonmedical therapy for restless legs syndrome includes : cold/warm compresses, warm/hot baths or showers, gentle massage, mild leg stretching at nighttime, or magnesium supplements (250- 1000 mg at nighttime daily). Mentally alerting activities help too. Note the caffeine, alcohol, nicotine, antidepressants, anti-nausea meds and antihistamines can cause or worsen symptoms.

- ECG due 7/2026
- UTOX due now - ordered
- in-person visit due 5/2026
- physician visit due 7/2026

Follow up in 3 months.

Kathleen E Moore, APRN.CNP

I spent a total of 40 minutes on the date of the service which included preparing to see the patient, face-to-face patient care, completing clinical documentation, obtaining and/or reviewing separately obtained history, counseling and educating the patient/family/caregiver, and ordering medications, tests, or procedures.

Recording using ambient AI software for draft documentation of the visit was discussed with the patient/ authorized representative; all questions welcomed and answered. Patient/authorized representative agreed to proceed.

[Note Details](#)

Instructions

Considerations when taking sodium oxybate

Side Effects

The most common side effects include nausea, headache, vomiting, dizziness, decreased appetite, weight decrease, headache, bedwetting and sleepwalking. Contact your healthcare provider if these occur.

Alcohol or Sedative Hypnotics

Do not drink alcohol or take other sedative hypnotics when you take oxybate salts.

Food Effects

Take the first dose at least 2 hours after eating.

Sedation

After taking oxybate salts you are likely to fall asleep quickly (often within 5-15 minutes), but the time it takes to fall asleep can vary from night to night. The sudden onset of sleep, including in a standing position or while rising from bed, has led to falls complicated by injuries, in some cases requiring hospitalization. Stay in bed after taking dose

Breathing problems

Oxybate salts may cause slower breathing, trouble breathing, and/or short periods of not breathing during sleep (sleep apnea). Please who already have breathing or lung problems may have a higher chance of having breathing problems. Sleep apnea should be treated prior to starting oxybate salts.

Mental health problems

Contact a healthcare provider immediately if you develop depressed mood, markedly diminished interest or pleasure in usual activities, significant change in weight and/or appetite, psychomotor agitation or retardation, increased fatigue, feelings of guilt or worthlessness, slowed thinking or impaired concentration, or suicidal ideation.

Operating Hazardous Machinery

Until you are reasonably certain that oxybate salts do not affect you adversely (e.g., impair judgment, thinking, or motor skills) you should not operate hazardous machinery, including automobiles or airplanes. You should not do anything after that requires full wakefulness or that is dangerous for at least 6 hours after taking oxybate salts.



After Visit Summary[\(English Snapshot\)](#)

- Automatic Snapshot taken 11/7/2025

Additional Documentation

Flowsheets: [Patient-Reported Data](#),
[CCHS PDMP NARXCARE SCORES](#)
SmartForms: [PHYSICIAN / PATIENT LOCATION ADVANCED](#)
Encounter [Billing Info](#),
Info: [History](#),
[Allergies](#),
[Detailed Report](#),
[Procedural Documentation](#)

Communications[View All Conversations on this Encounter](#)[Chart Routed to S3 Sleep Schedulers Pool](#)**Media**

From this encounter

[PSR PBB Medicare - MyChart - Electronic signature on 11/7/2025 1:47 PM \(effective from 11/7/2025\) - E-signed](#)**Pharmacy Benefits**[Verify Pharmacy Benefits](#) No pharmacy benefits eligibility data found for this visit.**Travel Screening and History****Disease Screening**

Row Name	11/07/25 1347
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Has the Patient	No 🗑️
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Had 2 Falls in the	
--------------------	--

Last Year or 1	
----------------	--

Fall with Injury or	
---------------------	--

Currently Using	
-----------------	--

an Ambulatory	
---------------	--

Assistive Device	
------------------	--

(Walker, Cane,	
----------------	--

Wheelchair,	
-------------	--

Crutches, etc.)	
-----------------	--

Are you having	No 🗑️
----------------	-------

pain associated	
-----------------	--

with your visit	
-----------------	--

today?	
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Encounter Report[Facesheet Report](#)**Orders Placed**

[FERRITIN](#)
[IRON AND TIBC](#)
[TOXICOLOGY SCREEN, ROUTINE URINE](#)

Medication Changes

None

[Medication List](#)

Visit Diagnoses



Hypersomnia due to medical condition

Long-term current use of stimulant

Restless legs syndrome

Disorder of iron metabolism

High risk medication use

[Problem List](#)

The following is a continuation of today's Clinical Medical Note:

Note that the following is the continuation of today's clinical staff physician note and that the above attached Physician/Advanced Practicing Nurse clinical note or Sleep Study report is for review purposes.

Pt says he's doing "okay." Pt says his sleepiness is more or less the same. Pt will still have good days and bad days, which are random. Pt says the times of day he is sleepy are completely random, though he says he'll never be able to work again. Pt is being tapered off of his psychiatric medications and is doing well with that.

Pt is currently prescribed Lumryz 6 g QHS, gabapentin 600 mg QHS, Adderall 10 mg BID, and Adderall XL 15 mg every day. In the past, pt was prescribed Ritalin and said that worked pretty well for him, though pt says his narcolepsy wasn't as severe back then.

PATIENT-ENTERED QUESTIONNAIRE SLEEP SCORES

11/7/2025	
Sleep Questions	
Reason for visit:	Narcolepsy Restless Legs Syndrome

Multiple values from one day are sorted in reverse-chronological order

	7/26/2024	7/24/2025	11/7/2025
Epworth Sleepiness Scale			
Score	Incomplete	4 (No clinically significant daytime sleepiness)	Incomplete

	7/26/2024	7/24/2025	11/7/2025
PROMIS CAT Sleep Disturbance			

	7/26/2024	7/24/2025	11/7/2025
PROMIS Sleep Disturbance T-Score	63 (moderate) !	47 (within normal limits)	Incomplete
PROMIS Sleep Disturbance Percentile	10	62	

	4/5/2024	7/26/2024	7/24/2025
Restless Leg Syndrome			
Score	Incomplete	Incomplete	30 (Severe symptoms) !

	4/5/2024	7/26/2024	7/24/2025
PHQ-9			
Score	8	7	0

Data saved with a previous flowsheet row definition

	2/17/2017	2/6/2019	7/24/2025
PROMIS Global Health - (T-Scores - the mean of general population = 50. Five points is a clinically meaningful difference.)			
Physical T-Score	39.8	50.8	50.8
Mental T-Score	43.5	48.3	50.8

PMH, PSH, SH: as above

SLEEP RELATED ROS

Review of Systems

Allergies

Allergen

- Penicillins

Had hives and wheezing

Reactions

Hives

CURRENT MEDICATIONS:

• insulin aspart, niacinamide, (FIASP FLEXTOUCH U-100 INSULIN) 100 unit/mL (3 mL) pen	Inject 15 units with meals and sliding scale 3
• BASAGLAR KWIKPEN U-100 INSULIN 100 unit/mL (3 mL)	Give 2x daily 65 units twice daily
• tirzepatide (MOUNJARO) 10 mg/0.5 mL pen injector	Inject 10 mg subcutaneously one time a week.
• sodium oxybate ER (LUMRYZ) 6 gram granules in packet	Take 1 packet by mouth daily at bedtime.
• hydrALAZINE (APRESOLINE) 25 mg tablet	Take 2 tablets by mouth once daily as needed (in the evening if SBP>150).
• gabapentin (NEURONTIN) 100 mg capsule	Start at 100 mg 2 hours before onset of RLS symptoms, Increase by 100 mg every 5 days until reaching relief or side effect or maximum of 600 mg

• ropinirole ER (REQUIP XL) 2 mg tablet	Take 2 mg by mouth daily at bedtime.
• dextroamphetamine/amphetamine (ADDERALL ORAL)	Take 10 mg by mouth two times a day.
• dextroamphetamine/amphetamine (ADDERALL XR ORAL)	Take 15 mg by mouth once daily.
• aspirin, enteric coated (ASPIRIN, ENTERIC COATED) 81 mg EC tablet	Take 81 mg by mouth once daily.
• cyclobenzaprine HCl (CYCLOBENZAPRINE ORAL)	Take 10 mg by mouth every 8 hours. Every 8 hours as needed
• ergocalciferol, vitamin D2, (DRISDOL ORAL)	Take 1.25 mg by mouth one time a week.
• GUAIFENESIN ORAL	Take 10 mL by mouth as needed.
• loperamide HCl (IMODIUM A-D ORAL)	Take 2 mg by mouth as needed. Do not exceed 8mg in 24 hours.
• loperamide HCl (IMODIUM ORAL)	Take 4 mg by mouth as needed. Do not exceed 8mg in 24 hours.
• needles, insulin disposable (NOVOFINE 30 MISC)	8 mm as needed. For DM
• pantoprazole sodium (PROTONIX ORAL)	Take 40 mg by mouth once daily.
• ropinirole HCl (ROPINIROLE ORAL)	Take 2 mg by mouth daily at bedtime.
• ondansetron HCl (ZOFTRAN ORAL)	Take 4 mg by mouth every 6 hours as needed.
• amphetamine-dextroamphetamine XR (ADDERALL XR) 15 mg capsule	Take 1 capsule by mouth once daily for 90 days.
• dextroamphetamine-amphetamine (ADDERALL) 10 mg tablet	Take 1 tablet by mouth two times a day for 30 days.
• flash glucose sensor (FREESTYLE LIBRE 2 SENSOR) kit	For blood sugar checks 4 times daily. Takes 4 injections of insulin
• atorvastatin (LIPITOR) 80 mg tablet	Take 80 mg by mouth once daily.
• ezetimibe (ZETIA) 10 mg tablet	
• metFORMIN (GLUCOPHAGE) 1,000 mg tablet	Take 1,000 mg by mouth two times a day with meals.
• fenofibrate nanocrystallized (TRICOR) 145 mg tablet	
• omega-3 acid ethyl esters (LOVAZA) 1 gram capsule	
• paliperidone ER (INVEGA) 9 mg 24 hr tablet	
• ABILIFY MAINTENA 400 mg injection	Patient states he is on a lower dose of this medication , taking in pill form orally. Dose being slowly decreased eventually will be off this medication.
• divalproex DR (DEPAKOTE) 250 mg EC tablet	Take 1 tablet by mouth three times daily.
• acetaminophen (TYLENOL) 325 mg tablet	Take 2 tablets by mouth every 6 hours as needed.

PHYSICAL EXAMINATION:

Reminder that this is a two dimensional audio and visual medical visit.

Mental Status Examination:

General: No acute distress

Alertness: Alert

Orientation: Oriented to person, place and time

Eye contact: Good

Mental attitude: Positive

Historian: Good

Constitutional: Pleasant and cooperative

Speech Rate: Normal

Speech Tone: Normal

Speech Articulation: Normal
Speech Spontaneity: Normal
Bradyphrenia: None
Loose Associations: None
Thought Processes: Normal
Tangential: No
Circumstantial: No
Abstraction: Seems normal
Computation: Seems relatively normal
Suicidal thoughts: None reported
Homicidal thoughts: None
Hallucinations (Auditory, visual, gustatory): None
Delusions: None
EPS: None
AIM Scale: 0
Violent ideations: None
Paranoid thoughts: None exhibited
Guarded: No
Obsessions: None
Phobias: None
Judgement: Good
Insight: Good
Fund of Knowledge: Intact
Mood: Euthymic
Affect: Reactive

Physical Examination:

Note that no supplement oxygen is in use

No acute distress; consistent with age; the patient appears slightly sleepy at the time of the appointment

IMPRESSION:

Willis-ekbom syndrome (primary encounter diagnosis)

Restless legs syndrome

Long-term current use of stimulant

Hypersomnia due to medical condition

Narcolepsy without cataplexy (hcc)

PLAN:

Therefore in summary, after reviewing the patients medical record in the Epic electronic medical record system and interviewing the patient, the following is my professional recommendation: _

- Increase Adderall XR to 25 mg every day
- Continue Adderall 10 mg BID - refills sent
- Continue Lumryz 6 g QHS - refills sent
- Follow up in 3 months

Message me in one month regarding the dose change.

It was certainly a pleasure visiting with you today in the Cleveland Clinic Neurological Institute Sleep Disorders Center.

It is my privilege to be a part of your medical care.

Please schedule a follow up appointment with me or with one of our collaborating Advance Practice Nurse Sleep Medicine Nurse Practitioners (APNs) either in person at the Main Campus S Building or virtually via Zoom **in approximately 90 days**. The appointment telephone number for the Sleep Disorder Center is 216-636-5860. Note that MyChart may also be used to schedule your next appointment.

Remember to follow up with your other medical specialists and your primary care provider.

If you have any **further questions** for me regarding the diagnosis or recommendations today, please do not hesitate to **send us a MyChart message** or call my collaborating Care Coordinator nurse Eugene, RN, BSN at 216-444-2165 Extension #5.

I spent a total time of over 30 minutes on the date of the service on this case. This included preparing to see the patient by reviewing the medical record prior to examining the patient (known as "Pre Charting"), interviewing the patient face to face in the clinic or virtually via audio and video secure Cleveland Clinic technology, ordering appropriate medications/tests/procedures, completing clinical documentation of the visit, counseling and educating the patient/family/caregiver, communicating with other health care providers as well as general care coordination.

Note: This medical document was scribed in person during the medical visit by the Cleveland Clinic Medical Scribe Fellow, Ashton Sears. The note was subsequently reviewed, and edited if needed, by Dr. Michael Saribalas.

Michael G Saribalas, DO, CBSM, ABSM
Director, Sleep Medicine Fellowship Program
Clinical Assistant Professor, Cleveland Clinic Lerner College of Medicine
Adjunct Assistant Professor, Ohio University College of Medicine
Core ACGME Faculty
Chief Experience Officer, Sleep Medicine Center
Clinical Staff Physician, Sleep Medicine

Neurological Institute
Sleep Disorders Center

Department of Neurology
Department of Psychiatry

Main Campus
9500 Euclid Avenue
Mail Code S-73
Cleveland, Ohio
44195
US